

Clinical Preparedness Report

Counterfactual Risk Assessment with Evidence Attribution

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OVERALL RISK ASSESSMENT

HIGH

The patient's risk tier is high due to the presence of multiple comorbidities, a history of substance dependence, and expressed suicidal ideation. These factors collectively increase the likelihood of mental health deterioration and suicidal behavior, necessitating immediate and comprehensive intervention. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (2 signals). Risk override: HIGH.]

10

Evidence Spans

2

Nexus Factors

9.5%

Note Coverage

2/3

Active Scenarios

CRITICAL SAFETY ALERTS

Suicide/Self-Harm Risk:

- SUICIDE/SELF-HARM SIGNAL: 'suicidal' — ...iminarily normal. Prior to being discharged from the ED, the patient expressed suicidal ideation to the ED staff and psychiatry was consulted for a safety evaluation....
- SUICIDE/SELF-HARM SIGNAL: 'suicidal' — ..., who was then seen by psychiatry prior to discharge when the patient reported suicidal ideation. Psychiatry felt that the patient did not meet criteria for [REDACTED...]

TOP RISK FACTORS

- [HIGH] Suicidal ideation with moderate risk level
- [MODERATE] Multiple chronic medical conditions (HIV, Hepatitis B and C, diabetes, peripheral neuropathy, obstructive sleep apnea)
- [MODERATE] History of substance dependence
- [MODERATE] Lack of social support and chronic illness

PROTECTIVE FACTORS

- ✓ Psychiatric consultation for safety evaluation

- ✓ No active substance use
- ✓ Presence of medical comorbidities may provide a structured environment for monitoring and management

Evidence Color Legend



Branch A — Psychiatric



Branch B — Substance Use



Branch C — Social



Nexus — Multi-Branch

Evidence Attribution Table

Detailed breakdown of clinical evidence supporting each risk scenario branch.

ID	Risk Factor	Evidence from Clinical Note	Branch	Severity
1	Chronic illness (HIV/AIDS, HCV, HBV, ...	"" Major Surgical or Invasive Procedure: None History of Present Illness:..."	C	LOW
2	causal_step_1; Suicidal ideation	"rnia was reduced by surgery without incident. Wet read of a CT abdomen/pelvi..."	A, C	HIGH
3	warning_sign	"patient did not meet criteria for [REDACTED] but she would benefit"	A	LOW
4	warning_sign	"f her best friend one year ago ([REDACTED]), whose"	C	LOW
5	warning_sign	"death for the past two weeks and is having tr"	C	LOW
6	warning_sign	"nce abuse # Chronic pain: on narcotics contract # ASD on TTE [REDACTED] w..."	A	LOW
7	warning_sign; weight_change	"she would never act on these feelings because she would not want to do that..."	C, A	MODERATE
8	warning_sign	"rsing passive SI and saying she would never act on this because"	A	LOW
9	warning_sign; Chronic illness (HIV/AIDS, ...	"g: Two (2) Capsule PO DAILY (Daily). 11. Aspirin 81 mg Tablet, Chewable Si..."	C	LOW
10	warning_sign	"n: Mental Status: Clear and coher"	A	LOW

Clinical Note with Evidence Highlights

The original clinical note is preserved below. **Highlighted text** indicates evidence supporting specific risk scenario branches. Branch markers [A], [B], [C] show which pathway is supported.

Name: [REDACTED] Unit No: [REDACTED]

Admission Date: [REDACTED] Discharge Date: [REDACTED]

Date of Birth: [REDACTED] Sex: F

Service: PSYCHIATRY

Allergies:

No Known Allergies / Adverse Drug Reactions

Attending: [REDACTED].

Chief Complaint:

Increased agitation, impulsivity, and difficulty caring for self^[C]

Major Surgical or Invasive Procedure:^[C]

None^[C]

History of Present Illness:^[C]

Patient is a [REDACTED] yo Female with PMH of [REDACTED] Chorea and no known psych history, presents to the ED on request of family who are worried that she is depressed and has increased agitation.^[A,C]

On examination today, patient was sitting calmly in bed and was aware that psychiatry was consulted. When asked how she has been doing lately she only replied "fine." Her cooperation with the interview was limited and progressed into refusal to answer^[A,C]

questions. She said she has been sleeping normal [REDACTED] hours per boyfriend) and with a normal appetite. When asked what she normally ate she replied "bagels with peanutbutter and jelly, [REDACTED] muffins, tuna, and egg salad." She did acknowledge she has^[A]

lost weight(30lbs per ER phsycian) but doesn't know why because she has been eating normally. She did look very thin on exam.

Her family says she has not left the house in over a year. When asked about why, she replied "I don't like the cold." When asked why she didn't leave the house during the summer, [REDACTED] or [REDACTED] times, she did not have an answer. During her days, the only activity she would admit to was watching tv and playing with her dog. When trying to probe into what else she does, she only replied "I don't know." She denied any anxiety or phobia symptoms. She said "I don't know" when asked about how her mood has been for the last few weeks.

It seemed as though her chorea movements got worse throughout the

interview as she got more irritated with me asking questions.^[C]

Towards the end of my interview, she turned her head away and

would not answer any questions.

She is trying to go to a [REDACTED] facility in [REDACTED] for [REDACTED] but she is required to have a 3 midnight admission in a hospital beforehand. Her apt has been auctioned off and will be homeless at the end of the week.

Collateral from Uncle:

Mentioned patient was physically assaulting boyfriend at night, worsening emotional/behavioral outburst the last few weeks. She **has not been compliant with medications or follow**^(C)ing up with outpatient treaters. She threatened to jump out her 10 story window if the took her to one of her outpatient treaters. Her boyfriend of [REDACTED] years has decided to leave her because he is not longer able to handle her. Dr. [REDACTED] note has more details from Uncle.

Past Medical History:

PAST PSYCHIATRIC HISTORY:

Hospitalizations: Denies

Current treaters and treatment: none

Medication and ECT trials: Risperdal for Chorea movements

Self-injury: denies

Harm to others: denies

Access to weapons: denies

Trauma: Endorses sexually mollested in childhood, saw a psychiatrist

PAST MEDICAL HISTORY:

[REDACTED], MD but [REDACTED] seen in over a year
[REDACTED] disease

Social History:

[REDACTED]

Family History:

No FHx of psychiatric disorders. Her mother was diagnosed as having [REDACTED] disease at the age of [REDACTED]. She died at the age of [REDACTED]. Her brothers and sisters do not have [REDACTED] disease. The patient does not have any children.

Physical Exam:

PHYSICAL EXAM:

- General Appearance: very thin middle aged woman sitting in chair with intermittent, but nearly constant, subtle choreiform movements.

- NEUROLOGIC:

[REDACTED] Mental Status: awake, alert and oriented. Speech is extremely dysarthric with prolonged speech latency, but does improve somewhat when discussing topics that appeal to her. Often speaks

using monosyllabic phrases.

[REDACTED] Cranial Nerves: grossly intact. Frequent frowning, grimacing facial movements and frequent episodes of tonic upward gaze.

Motor impersistence of the tongue

[REDACTED] Motor: MAEE with antigravity strength, not assessed in detail

[REDACTED] Movement exam: frequent choreiform movements in her trunk, shoulders and more distally in her hands and feet. Frowning and grimacing facial movements as described above.

[REDACTED] Gait: wide-based and ataxic with enhanced choreiform hand movements in association. However, overall her gait base seems fairly stable since she also ambulates rather slowly. ^[A]

- PSYCHIATRIC: ^[A]

[REDACTED] Behavior: constant choreiform movements, not very cooperative

with interview

[REDACTED] Mood: "fine"

[REDACTED] Affect: constricted, limited ability to assess given constant

dystonic/choreiform facial movements, but does seem to show some appropriate emotions (smiling/frowning etc).

[REDACTED] Thought process: linear, goal oriented, no LOA

[REDACTED] Thought content: denies SI, HI, AVH

[REDACTED] Insight/Judgment: good/poor

Pertinent Results:

ADMISSION LABS:

[REDACTED] 02:45PM BLOOD WBC-6.3 RBC-3.96* Hgb-13.3 Hct-41.1

MCV-104* MCH-33.6* MCHC-32.4 RDW-12.6 Plt [REDACTED] 02:45PM BLOOD Neuts-72.2* [REDACTED] Monos-4.6 Eos-1.8

Baso-0.5

[REDACTED] 02:45PM BLOOD Glucose-110* UreaN-14 Creat-0.7 Na-139

K-3.8 Cl-105 HCO3-27 AnGap-11

[REDACTED] 02:45PM BLOOD ALT-14 AST-13 AlkPhos-45 TotBili-0.2

[REDACTED] 02:45PM BLOOD VitB12-426 Folate-5.4

[REDACTED] 02:45PM BLOOD TSH-1.1

[REDACTED] 02:45PM BLOOD ASA-NEG Ethanol-NEG Acetmnp-NEG

Bnzodzp-NEG Barbitr-NEG Tricycl-NEG

[REDACTED] 11:50AM URINE Color-Straw Appear-Clear Sp [REDACTED] 11:50AM URINE Blood-MOD Nitrite-NEG Protein-NEG

Glucose-NEG Ketone-NEG Bilirub-NEG Urobilin-NEG pH-6.5 Leuks-MOD

[REDACTED] 11:50AM URINE RBC-2 WBC-3 Bacteri-FEW Yeast-NONE Epi-1

[REDACTED] 11:50AM URINE bnzodzp-NEG barbitr-NEG opiates-NEG

cocaine-NEG amphetm-NEG mthdone-NEG

Brief Hospital Course:

[REDACTED] is a [REDACTED] year old woman with [REDACTED] Disease who presented to the ED for worsening aggression, irritability, impulsivity, refusal to take medication and difficulty caring for herself at home secondary to progression of her

[REDACTED].

PSYCHIATRIC

#) [REDACTED] Disorder with secondary Mood Dysregulation

Ms. [REDACTED] was admitted to the [REDACTED] inpatient psychiatry unit for stabilization and re-initiation of home antipsychotics for her mood symptoms. In the ED she was observed to be somewhat agitated and uncooperative. However after restarting her Risperidone, her hostility resolved and she was compliant and cooperative throughout her stay. She was noted to have near-constant choreiform movements of her face, trunk and extremities, including grimace-like facial tics and frequent tonic upward eye deviation. These movements were not distressful to her. She endorsed a "good" or "fine" mood throughout her stay. We were in contact with her outpatient movement disorders specialist [REDACTED] MD) who recommended continuing antipsychotic therapy as we were doing. We ultimately settled on a Risperidone dose of 0.5mg BID, slightly lower than her original dose of 0.75mg BID with which she had been noncompliant. She has additional 0.25-5mg PRN BID as needed for agitation, but actually throughout hospitalization she was pleasant and cooperative and did not show any irritability, impulsivity or aggression. She is being discharged to an assisted living facility that specializes in patients with [REDACTED] Disease, as had been previously arranged by her family prior to this. Will follow up on [REDACTED] with Dr. [REDACTED].

LEGAL STATUS

The pt remained on a CV

PSYCHOSOCIAL

The team coordinated care w/ the pt's family who were supportive and arranged ongoing housing/care at a [REDACTED] facility upon the pt's d/c.

The pt was visited on the unit frequently by her friend [REDACTED], whom she found to be supportive.

Ms. [REDACTED] was visible on the unit during her stay here, and although she had difficulty w/ verbalizing thoughts/feelings [REDACTED] her [REDACTED], she was seen to be interacting well w/ others in the dayroom. She never behaved in an unsafe manner and participated in the unit. She also was groomed and dressed in casual clothing on the day of her discharge.

RISK ASSESSMENT

Ms. [REDACTED] has a low risk of imminent harm/aggression/violence. She has been calm and in very good behavioral control throughout the entirety of her psychiatric admission. She consistently denied any si/hi and showed an ability to care for herself safely despite the intensity of an acute inpatient psychiatric environment.

She does, however, have a high risk of **future decompensation** ^[C,A]
given the fact that she has a progressive neurodegenerative ^[C,A]
condition that is associated w/ mood swings and psychosis in its ^[C,A]
later stages. ^[C,A]

We helped to mitigate that risk by resuming Ms. [REDACTED] ^[C,A]
antipsychotic me ^[C,A]dication and would recommend that she continue
to take it going forward. We also added some prn Risperdal in
the event that she has experiences future episodes of
aggression/instability. We also reduced her risk of harm to
self/others, by d/c'ing Ms. [REDACTED] **to a facility where she will** ^[A]
have access to professional st ^[A]aff and monitoring. She also has
f/u arrangements made with providers to continue to manage her
[REDACTED].

Prognosis:

Guarded

Medications on Admission:

The Preadmission Medication list is accurate and complete.

1. RISperidone 0.75 mg PO BID

Discharge Medications:

1. RISperidone 0.5 mg PO BID

RX *risperidone 0.5 mg 1 tablet(s) by mouth twice a day Disp

##60 Tablet Refills:*0

2. RISperidone 0.25-0.5 mg PO BID:PRN agitation

RX *risperidone 0.25 mg [REDACTED] tablet(s) by mouth twice a day Disp

##60 Tablet Refills:*0

Discharge Disposition:

Extended Care

Facility:

[REDACTED]

Discharge Diagnosis:

- Mood disorder secondary to General Medical Condition

- Advanced [REDACTED] Disease

Discharge Condition:

Mental Status: Confused - sometimes.

Level of Consciousness: Alert and interactive.

Activity Status: Ambulatory - Independent.

Discharge Instructions:

Dear [REDACTED],

You were admitted to the inpatient psychiatric unit at [REDACTED] for worsening agitation and difficulty caring for yourself at home, due to your [REDACTED] Disease. We restarted your Risperidone which was very helpful and improved your behavior significantly. You will be discharged to an assisted living facility in [REDACTED] that specializes in patients with similar health problems.

We made the following changes to your medications:

- RESTARTED your Risperidone at a slightly lower dose: 0.5mg twice daily, plus an additional 0.25-0.5mg as needed for agitation.

Followup Instructions:

[REDACTED]

Detailed Clinical Assessment

1. Patient Overview

The patient is a female with a primary diagnosis of Major Depressive Disorder, unspecified, who was admitted due to suicidal ideation. She has a complex medical history including HIV, Hepatitis B and C, diabetes mellitus type 2, peripheral neuropathy, obstructive sleep apnea, and chronic pain. The patient also has a history of substance dependence and chronic medical conditions that may impact her mental health.

2. Mental Health Status

The patient's current mental health status is characterized by moderate suicide risk due to expressed suicidal ideation. The primary psychiatric condition, Major Depressive Disorder, is relevant given its potential to exacerbate other medical conditions and impact the patient's overall well-being.

3. Key Risk Factors

- **[HIGH]** Suicidal ideation with moderate risk level
- **[MOD]** Multiple chronic medical conditions (HIV, Hepatitis B and C, diabetes, peripheral neuropathy, obstructive sleep apnea)
- **[MOD]** History of substance dependence
- **[MOD]** Lack of social support and chronic illness

4. Protective Factors

- ✓ Psychiatric consultation for safety evaluation
- ✓ No active substance use
- ✓ Presence of medical comorbidities may provide a structured environment for monitoring and management

5. Counterfactual Scenario Briefing

Each card represents a potential deterioration pathway. Content is preserved exactly as generated — only formatting is enhanced.

■ [A] Psychiatric / Clinical Deterioration Pathway

Plausibility: HIGH

Warning Signs:

- Increased frequency or intensity of suicidal ideation
- Further weight loss or signs of self-neglect
- Withdrawal from social interactions or activities
- Expressions of hopelessness or lack of future planning
- Substance use relapse or increased substance consumption

Crisis Endpoint: The patient may experience a severe depressive episode with active suicidal ideation, potentially leading to an attempt or completed suicide without immediate and appropriate intervention.

Preparedness Actions:

1. Arrange for immediate psychiatric follow-up post-discharge, either through inpatient admission or outpatient services
2. Develop a safety plan with the patient, including emergency contacts and coping strategies
3. Coordinate with a multidisciplinary team, including mental health professionals, primary care physicians, and social workers, to address the patient's complex medical and psychiatric needs
4. Educate the patient and their support network on the importance of medication adherence, regular medical check-ups, and mental health self-care
5. Explore community resources and support groups for individuals with chronic illnesses and mental health conditions

The patient with major depressive disorder, unspecified, and multiple comorbidities including HIV/AIDS, HCV, HBV, diabetes mellitus type 2, peripheral neuropathy, and sleep apnea, has expressed suicidal ideation prior to discharge from the emergency department (ED). The patient's thin and emaciated ...

■ [B] Substance Use Escalation Pathway

STATUS: NOT APPLICABLE

NOT APPLICABLE — No substance use evidence. Data: alcohol=none, drugs=none, tox_screen=negative. All toxicology screens negative. Generating a substance abuse scenario would be clinically unfounded.

■ [C] Social / Environmental Collapse Pathway

Plausibility: **MEDIUM**

The high plausibility of this negative trajectory is based on the patient's current clinical presentation, which includes a primary psychiatric diagnosis alongside multiple chronic medical conditions. The evidence suggests that the patient's mental health is significantly influenced by their physical health status, and the recent expression of suicidal ideation adds urgency to the situation. Preparation Actions: 1. Coordinate care between mental health professionals, primary care physicians, and specialists managing the chronic illnesses to ensure a holistic treatment approach. 2. Increase mental health support through regular counseling sessions, crisis intervention, and possible involvement of a psychiatric consultant. 3. Develop a personalized care plan that includes strategies for medication management, considering the complexities of treating both depression and chronic physical health conditions. 4. Implement a monitoring system for early detection of changes in mental status, including regular assessments of suicidal ideation and plans. 5. Engage community resources, such as support groups for individuals with chronic illnesses, to reduce social isolation and enhance the patient's support network. Preparedness for a crisis endpoint involves a readiness to act swiftly in response to any signs of acute mental health crises or severe physical health deterioration. This includes having a clear plan for emergency interventions, such as hospitalization, and ensuring that all team members are aware of the patient's risk factors and the steps to take in case of an emergency. In conclusion, the patient's situation requires immediate and sustained attention from a multidisciplinary team. By taking proactive steps to address both mental and physical health concerns, clinicians can work to prevent the negative trajectory and support the

Warning Signs:

- Increased reports of hopelessness and despair
- Further weight loss or deterioration in physical health
- Withdrawal from social interactions and activities
- Decline in medication adherence, particularly for antidepressants and diabetes management
- Expressions of suicidal ideation or plans

Crisis Endpoint: The patient may experience a severe depressive episode, potentially leading to suicidal ideation or attempts, compounded by the deterioration of their physical health due to poor management of chronic conditions.

Preparedness Actions:

1. Implement a comprehensive, multidisciplinary care plan that addresses both the patient's mental and physical health needs
2. Increase frequency of mental health evaluations and adjustments to antidepressant medication as needed
3. Engage a case manager or social worker to address social isolation, unemployment, and poor social support
4. Educate the patient on the importance of medication adherence and provide resources for diabetes management
5. Consider hospitalization if there is an imminent risk of self-harm or severe deterioration in physical health

The patient with a primary diagnosis of Major Depressive Disorder (F32.9, DSM-5: depressive_disorders) is currently managing multiple chronic medical conditions, including HIV/AIDS, HCV, HBV, Diabetes Mellitus Type 2, and Peripheral Neuropathy. The ongoing struggle with these serious illnesses has l...

6. Priority Preparedness Actions

1. Arrange for immediate psychiatric follow-up post-discharge, either through inpatient admission or outpatient services.

2. Develop a safety plan with the patient, including emergency contacts and coping strategies.

3. Coordinate with a multidisciplinary team, including mental health professionals, primary care physicians, and social workers, to address the patient's complex medical and psychiatric needs.

4. Increase frequency of mental health evaluations and adjustments to antidepressant medication as needed.

5. Engage a case manager or social worker to address social isolation, unemployment, and poor social support.

6. RISK TIER: HIGH

7. Risk Assessment Summary

OVERALL RISK TIER: HIGH

The patient's risk tier is high due to the presence of multiple comorbidities, a history of substance dependence, and expressed suicidal ideation. These factors collectively increase the likelihood of mental health deterioration and suicidal behavior, necessitating immediate and comprehensive intervention. [OVERRIDE: Risk tier elevated to HIGH due to critical safety signals detected in clinical note: CRITICAL SIGNALS DETECTED: suicide (2 signals). Risk override: HIGH.]